

Child Poverty in Zimbabwe

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for every child

A Multiple Overlapping
Deprivation Analysis



Policy Brief



August 2021

1. Why Measure Multidimensional Child Poverty?

Like many countries in Africa, Zimbabwe has a relatively young and rapidly growing population. At the national level, 79 per cent of all households have at least one child under the age of 18 and 53 per cent have at least one child under the age of five (MICS6, 2019). Among Zimbabwean children, 61 per cent and 36 per cent are income and food poor, respectively, with children living in rural areas experiencing higher levels of poverty (ZIMSTAT and UNICEF, 2019a). There is a need to understand child poverty in its complexity, to effectively address child vulnerabilities and work towards reducing deprivations. In this policy brief, evidence is presented on the multiple and overlapping deprivations facing children in Zimbabwe. This evidence aims at informing policies and the subsequent implementation of adequate interventions.

This policy brief presents data to monitor and assess Zimbabwe's progress in achieving development for children as per the Sustainable Development Goal (SDG) 1.2. The objective of SDG 1.2 is to reduce at least by half the proportion of children of all ages living in poverty in all its dimensions according to national definitions.

2. How is Multidimensional Child Poverty Defined and Measured in Zimbabwe?

The analysis employed by this policy brief is based on UNICEF's multiple overlapping deprivation analysis (MODA) (Neubourg et al., 2013). This methodology is designed to fit the needs of children and to stimulate the design of child-sensitive social protection (Neubourg et al., 2018; Neubourg et al., 2021).

Multidimensional poverty among children is measured by the following dimensions of child well-being: nutrition, health, child protection, development (cognitive), water, sanitation, housing and information (see Table 1). Each of these dimensions of well-being is composed of a set of indicators (see Annex 1 in full report). The selection of indicators and dimensions for this study is dependent on the Multiple Indicator Cluster Survey (MICS) 2019 data set, and follows the importance of measuring the specific needs of children in the context of Zimbabwe. Furthermore, the choice of parameters relies on the Convention on the Rights of the Child (CRC) and the SDGs, as international guidelines to measure multidimensional child poverty. The final selection of indicators and dimensions was agreed upon by UNICEF and national stakeholders in the country.

Considering the life-cycle approach, the dimensions are disaggregated into four age groups (0–23 months, 24–59 months, 5–14 years, and 15–17 years) to better reflect the needs of children and their development at different stages of childhood.

Table 1: List of dimensions measuring multidimensional child poverty in Zimbabwe

0-23 months	24-59 months	5-14 years	15-17 years
Nutrition	Nutrition	Education	Education
Health	Child Protection	Child Protection	Water
Child Protection	Development	Water	Sanitation
Water	Water	Sanitation	Housing
Sanitation	Sanitation	Housing	Information
Housing	Housing	Information	
Information	Information		

3. What are the Levels of Multidimensional Child Poverty in Zimbabwe?

A child is considered to be multidimensionally poor in Zimbabwe if he/she suffers simultaneously from deprivation in at least three dimensions of well-being. Based on this definition, 60.7 per cent of all children in the country are multidimensionally poor. The disaggregation of results by age groups shows that 89.3 per cent of children aged 0–23 months, 75.1 per cent of children aged 24–59 months, 54.5 per cent of children aged 5–14 years and 44.8 per cent of children aged 15–17 years are multidimensionally poor (see Figure 1).

Almost all children in Zimbabwe (96.7%) suffer from at least one deprivation, while 84.1 per cent experience at least two deprivations at the same time (see Figure 2). Approximately 14.7 per cent of children are simultaneously deprived in five or more dimensions¹.

¹The number of total possible deprivations varies across age groups due to data availability (Table 1). A total of 7 dimensions was included for children under the age of 5, 6 dimensions for children aged 5–14 years, and 5 dimensions for children aged 15–17 years. In the count of multidimensional deprivation, children with 6 and 7 deprivations are those up to the age of 14.

Figure 1: Multidimensional poverty by age group

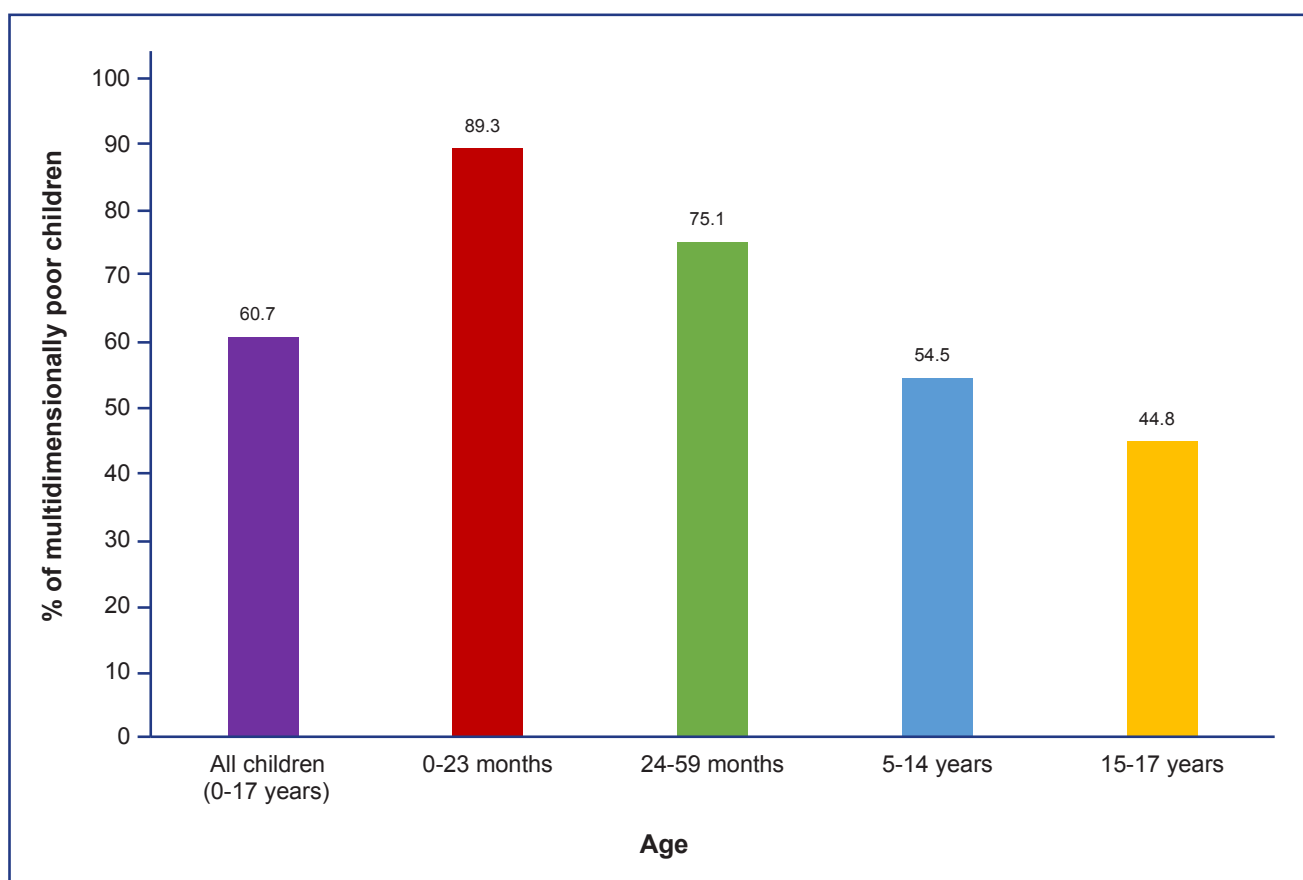
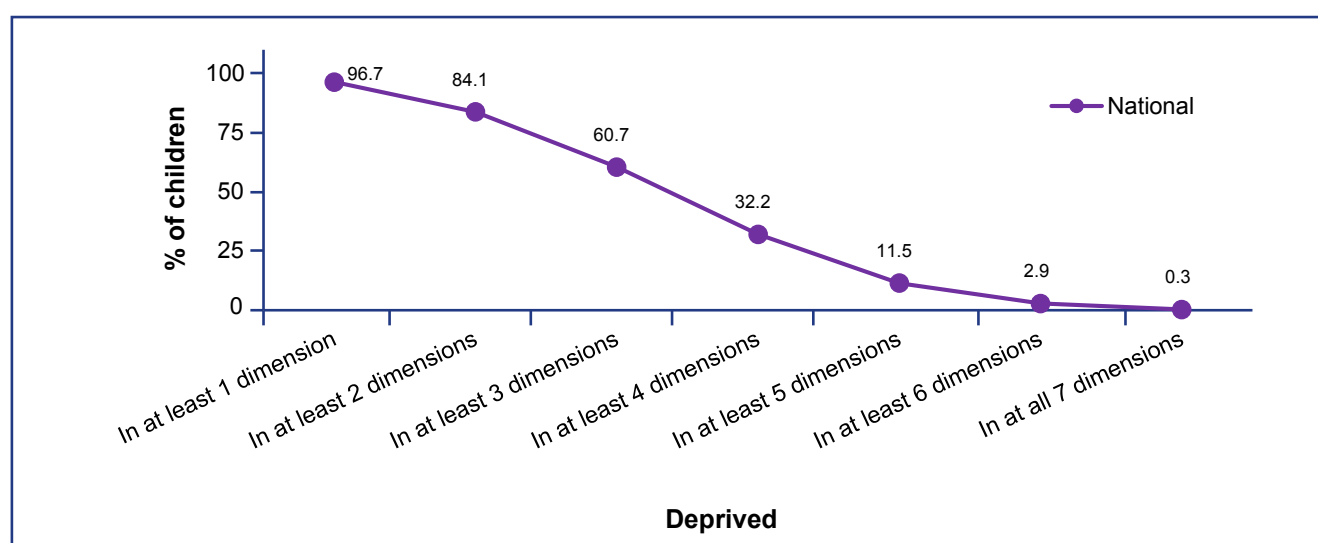


Figure 2: Percentage of children that suffer deprivation in at least one dimension



4. In Which Dimensions are Children Most Deprived?

In Zimbabwe, the highest levels of deprivations are found in the area of sanitation (77%–85%) for all age groups, except for children aged 24–59 months, where the highest deprivation rate is found in the dimension of protection (84%) (see Table 2). The lowest rates of deprivation are observed in information (7%–9%), while approximately half of the children across all age groups are deprived in the water dimension (50%).

Table 2: Percentage of children deprived in each dimension of their well-being, by each age group

0–23 months	24–59 months	5–14 years	15–17 years
<i>Sanitation: 85.2%</i>	<i>Protection: 83.8%</i>	<i>Sanitation: 81.1%</i>	<i>Sanitation: 77.2%</i>
<i>Protection: 79.6%</i>	<i>Sanitation: 82.0%</i>	<i>Housing: 63.1%</i>	<i>Housing: 56.8%</i>
<i>Nutrition: 77.6%</i>	<i>Housing: 64.3%</i>	<i>Water: 53.1%</i>	<i>Water: 49.8%</i>
<i>Housing: 65.5%</i>	<i>Water: 50.6%</i>	<i>Protection: 47.6%</i>	<i>Education: 39.8%</i>
<i>Health: 59.4%</i>	<i>Development: 25.9%</i>	<i>Education: 9.1%</i>	<i>Information: 7.7%</i>
<i>Water: 50.7%</i>	<i>Nutrition: 25.3%</i>	<i>Information: 8.7%</i>	
<i>Information: 8.2%</i>	<i>Information: 7.3%</i>		

5. Where are the Most Deprived Children?

The disaggregation of poverty among rural and urban areas shows major disparities in the country, where 69.2 per cent of rural children are multidimensionally poor compared to 37.6 per cent of urban children. Of all regions, **Bulawayo (37%)**, **Harare (39%)** and **Mashonaland East (56.3%)** have the lowest proportion of multidimensionally poor children in the country. The province of Matabeleland North, on the other hand, has the highest rate of multidimensional child poverty, as more than 7 out of 10 children are multidimensionally poor (73.4%).

6. What Factors are Driving Child Poverty the Most?

The findings show that multidimensionally poor children tend to live in households with some particular characteristics. For instance, multidimensional poverty is higher among children living in households with more members. For example, 61.2 per cent of children living in households with 5 or more members are multidimensionally poor as compared to 49.8 per cent of children in households with 1–2 members. Similarly, the higher the number of children

in the household, the higher the level of multidimensional child poverty. Furthermore, children under the age of five are more likely to be multidimensionally poor when living in households that experienced at least one case of child mortality in the last five years (72.3%) as opposed to children in households with no child mortality (63.8%).

Multidimensional child poverty is slightly higher among children living in female headed households (62.4%) as compared to children living in male headed households (59.6%). Up to 71 per cent of all children whose household heads have no education, pre-primary or primary education are multidimensionally poor compared to 53 per cent of children whose household heads attained secondary or higher education. It is important to note that multidimensional child poverty is present in one out of two households even if the household head achieved at least secondary education. Households with labour constraints are also worse off when it comes to multidimensional child poverty when compared to households which are not labour constrained (67.5% versus 57.2%, respectively).

The education level of the mother plays an important role with regard to multidimensional child poverty. The child poverty rate is as high as 78.3 per cent among children whose mothers attained no education, pre-primary or primary education as opposed to 54.9 per cent of children with mothers who achieved middle, secondary or higher education. The education level of the father could not be employed in the analysis due to methodological limitations.

In terms of living arrangements, it is found that multidimensional child poverty is higher for children not living with their parents compared to children living with at least one biological parent (62.6% versus 54.7%, respectively). The marital status of the mother does not present differences when it comes to multidimensional child poverty. The analysis did not observe any gendered differences in the multidimensional poverty of boys and girls in Zimbabwe.

7. Multidimensional Poverty versus Monetary Poverty

Children who are multidimensionally poor are not always monetarily poor and vice versa. The analysis makes a distinction between monetary² and multidimensional poverty when assessing child vulnerability.

At national level, there is only a slight difference between the two rates with 60.7 per cent of children being multidimensionally poor while 61.3 per cent are monetarily poor. However, larger disparities are observed at the rural/urban level. In rural areas, monetary poverty (76.3%) is higher than multidimensional child poverty (69.4%). On the other hand, in urban areas, multidimensional child poverty (38%) is higher than monetary poverty (20%).

²A child is defined as monetarily poor if that child lives in a household where its members are living under the national poverty line. The calculations for monetary poverty are based on the Poverty, Income, Consumption and Expenditure Survey 2017.Z

8. Trends in Child Poverty between 2014 and 2019

In 2016, a multidimensional poverty measure for children was constructed based on MICS5 2014 data. The current study conducted an analysis with MICS6 2019 using the exact same indicators and dimensions as in the 2016 report (see Annex 2 in full report). One exception was made to exclude the indicator ‘Mother’s HIV/AIDS knowledge’ because of its absence in MICS6 2019.

The trend analysis shows that Zimbabwe made progress in reducing the multidimensional child poverty rate from 27.1 per cent in 2014 to 21.0 per cent in 2019. The intensity of deprivation has also decreased slightly over time, and the multidimensionally poor children experienced deprivations in a lower number of dimensions in 2019 compared to 2014. However, the progress in reducing child vulnerabilities varies across dimensions. While the Health, Development, and Education dimensions fared better in reducing child deprivations, the Water and Sanitation dimensions showed increased rates of vulnerability over time.

9. Strategic Policy and Programmatic Implications

Based on the findings of the study, the following policy recommendations are presented:

- **Adopt an intrasectoral approach for policies targeting children.** In Zimbabwe there are large overlaps between dimensions of well-being, which reflects negatively on the severity of deprivation among children. For example, 55.4 per cent of children aged 0–23 months face deprivation in the dimensions Nutrition, Sanitation and Child Protection simultaneously. In addition, 18.5 per cent of children aged 15–17 years are deprived in the dimensions Education, Water and Sanitation at the same time. Given the overlap, there is a need to look at multisectoral solutions to implement policy actions. Integrated cross-sectoral actions have a greater potential for reducing the multidimensional deprivation at different stages of a child’s life cycle.
- **Social and fiscal policies should target the most vulnerable children.** This study finds that certain groups of children are particularly vulnerable in Zimbabwe, with a high incidence of multidimensional poverty. Particular attention should be given to children living in rural areas and Matabeleland North, those living in large and labour constraint households, and those in household where heads and mothers have lower levels of education, among others. Interventions tailored to target these groups of children must be considered in the short to medium term. In addition, interventions should focus on nutrition and protection for younger children, and on improving housing, sanitation, and access to information for all children. Social protection

programmes with a 'cash plus' component are examples of such interventions (Neubourg et al., 2021), as they also integrate complementary services targeted at sectoral vulnerabilities. Ensuring that social protection programmes are based on country-specific evidence and regular monitoring will help to achieve a higher positive impact on the most vulnerable children and their families.

- **Improve data collection to create a robust framework for measuring and monitoring child well-being.** The Multiple Indicator Cluster Surveys (MICS) focus mostly on children younger than five years, such that the information on older children is limited. Available data on children are also collected from adults and are largely cross-sectional in design. These data limit the capacity to integrate child reports and voices in observing changes in child development across childhood. It is further recommended to **integrate longitudinal designs in data collection on children and families**. Similarly, large-scale surveys should include questionnaires aiming at child-specific responses when collecting data on children.

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